

Rehabilitation in Primary Care

On the gap between political acknowledgement and operational inclusion in primary health care.

"Rehabilitation is frequently referenced in the service continuum of UHC and primary health care documents. It is rarely defined as a required component of essential service packages or PHC team composition."

THE POSITION IN BRIEF

The World Rehabilitation Alliance holds that primary care is the critical site for rehabilitation integration — and that the gap between political mandate and operational delivery is the primary obstacle to equitable rehabilitation at scale. A coherent architecture of WHO, WHA, and UN commitments names rehabilitation within primary health care. Few countries have translated those commitments into operational inclusion. This statement sets out WRA's position on what operational inclusion requires.

WRA POSITION

Primary care is where rehabilitation needs first become visible to the health system, and the critical site through which rehabilitation must be delivered at scale. **The gap between political acknowledgement of rehabilitation within primary health care and its operational inclusion in essential service packages, PHC team composition, financing, and routine monitoring is the primary obstacle to equitable rehabilitation access.**

THE CASE FOR PRIMARY CARE AS THE CRITICAL SITE

Primary care is where rehabilitation needs first become visible to the health system. It is where timely intervention — whether for a person recovering from stroke, a child with developmental delay, an older adult experiencing functional decline, or a worker returning to productive life after injury — costs least and delivers most. Yet it remains the level at which rehabilitation is most systematically absent.

Sustainable Development Goal 3.8 calls for universal health coverage, which includes all essential services such as rehabilitation. Integrating funded rehabilitation into primary care — where most people first seek care — offers a strategic, scalable approach to expand access, promote equity and person-centred care, and enable systems to adapt to demographic shifts and emergencies.

2.6B

people globally live with a condition that could benefit from rehabilitation — 1 in 3

+63%

rise in rehabilitation needs since 1990, driven by ageing, NCDs, injuries, and mental health conditions

~50%

of those who need rehabilitation services globally cannot access them

Many of these needs are manageable at primary care level. When rehabilitation is absent from primary care, people are pushed toward specialised services they often cannot reach, experience delayed or incomplete recovery, and carry out-of-pocket costs that entrench poverty and exclusion — particularly in rural, remote, and conflict-affected settings.

THE MANDATE EXISTS — THE OPERATIONAL GAP IS THE PROBLEM

Rehabilitation within primary health care is named across a coherent architecture of WHO, WHA, and UN commitments. What is missing in most countries is not political mandate but operational inclusion. The distinction between what has been agreed politically and what is required for practical delivery is where WRA focuses its analysis and advocacy.

POLICY INSTRUMENT	WHAT IT MANDATES	WHAT OPERATIONAL INCLUSION REQUIRES
WHA76.6 (2023)¹	Calls for rehabilitation integration at all levels, from primary to tertiary	National essential service packages must define rehabilitation as a required component, not an optional add-on
WHA72.2 (2019)²	Affirms PHC as the foundation of health systems; defines scope of services, teams, and community-based delivery	PHC team composition standards must specify rehabilitation competencies and workforce

Declaration of Astana (2018)³	Affirmed rehabilitation as an essential service within primary health care	National PHC policies must translate this affirmation into benefit package inclusion and service delivery standards
WHO Global Rehabilitation Indicators (EB158/26, 2026)⁴	Provides a framework for tracking countries' governance, financing, workforce, and service delivery; includes "rehabilitation integrated into primary healthcare" as an output indicator, and an effective coverage indicator as the measure of health system performance	National health information systems must integrate these indicators to make rehabilitation visible; operational standards for what PHC integration requires are needed to measure against
UN Political Declarations on UHC (2019, 2023)^{5,6}	Affirm rehabilitation as an essential health service within UHC and the PHC approach	National UHC reform processes must include rehabilitation within the essential package, financed through pooled mechanisms
UN Political Declaration on NCDs and Mental Health (2025)⁷	Names rehabilitation in the continuum of care for NCD management	NCD care pathways in primary care must include rehabilitation as a standard component, not a referral to absent services
GPW14 (2025–2028)⁸	Positions rehabilitation across the full continuum of PHC-based system strengthening	WHO country-level implementation must ensure rehabilitation is operationally defined in PHC-strengthening plans

Primary care is where functioning first becomes visible to the health system.

Embedding rehabilitation at this level makes functioning — a third dimension of population health alongside mortality and morbidity — a routinely assessed outcome. What is routinely assessed can be tracked; what is tracked can be financed; what is financed can be delivered at scale. WRA holds that the measurement-financing sequence begins at primary care.

WHAT OPERATIONAL INTEGRATION LOOKS LIKE IN PRACTICE

✓ Brazil

Brazil's Family Health Strategy (Estratégia Saúde da Família) embeds rehabilitation within primary care through the Núcleos Ampliados de Saúde da Família (NASF) — multidisciplinary teams that explicitly include rehabilitation professionals alongside family physicians, nurses, and community health agents. This model defines rehabilitation as a required component of primary care team composition rather than a referral to absent services. It demonstrates that operational inclusion of rehabilitation within PHC is achievable at national scale — across diverse urban and rural contexts, and within a universal health system (Sistema Único de Saúde).⁹

Five principles for operational inclusion

WRA holds that operational inclusion of rehabilitation within primary care requires action across five interlocking dimensions. Each principle corresponds to a specific policy lever where the gap between mandate and delivery can be closed.

- 1 Rehabilitation must be defined as a required component of national essential service packages.**

Political acknowledgement translates into delivery only when rehabilitation is specified as a required component at primary care level, with associated financing, referral pathways, and minimum service standards. Optional or aspirational inclusion is insufficient.
- 2 PHC team composition and training standards must specify rehabilitation competencies.**

PHC workforce standards must include rehabilitation roles and competencies. Rehabilitation content must be embedded in medical, nursing, and allied health curricula. Primary care professionals must be equipped to identify rehabilitation needs, deliver foundational interventions, and refer appropriately.
- 3 Community health worker policy must integrate rehabilitation competencies.**

CHW frameworks are a major route for rehabilitation at community level, particularly in rural and underserved areas. Rehabilitation competencies must be integrated into CHW roles, with supervision structures linking community workers to the broader rehabilitation workforce.
- 4 Routine PHC monitoring must include rehabilitation service utilisation and functioning outcomes.**

The WHO Global Rehabilitation Indicators (EB158/26) provide a framework for Member States to track rehabilitation integration in primary care, service utilisation, and effective coverage. Integrating these indicators into national health information systems — alongside the WHO-UNICEF PHC monitoring framework — makes rehabilitation visible, and makes it possible to assess whether PHC systems are meeting population rehabilitation needs.
- 5 Public financing must reach primary care rehabilitation, reducing out-of-pocket expenditure and reliance on charitable provision.**

Rehabilitation services must be integrated into pooled financing mechanisms and public benefit packages. Reliance on philanthropic and charitable sectors must be reduced. Financing must reach rural, remote, and conflict-affected areas where access is weakest.

WRA'S POSITION

The World Rehabilitation Alliance holds that equitable rehabilitation access at scale cannot be achieved without operational inclusion of rehabilitation within primary health care. Political acknowledgement across WHO, WHA, and UN instruments has established the mandate. The operational gap — where rehabilitation is named in policy documents but not defined as a required component of essential service packages, PHC team composition, financing, or routine monitoring — is the primary obstacle to delivery.

WRA calls on Member States, WHO regional and country offices, primary care leaders, and global health financing actors to close this gap. The 2027 UN High-Level Meeting on UHC, the implementation of GPW14, and ongoing national UHC reform processes are critical windows for translating political acknowledgement into operational commitment.

From acknowledgement to inclusion.

A WRA Position Statement on Rehabilitation in Primary Care.

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